

Perform a HEENT Exam

081-833-0254

Conditions: You have a patient requiring a head, ears, eyes, nose and throat (HEENT) examination. All other apparent life threats have been treated and managed. You have performed a patient care handwash, taken body substance isolation (BSI) precautions. You will need an otoscope, disposable speculum attachments, tongue blades, pen light, pen, Snellen chart, and SF 600 (Medical Record-Chronological Record of Medical Care). You are not in a CBRN environment.

Standards: Perform an HEENT examination without causing further injury to the patient.

Performance Steps

1. Gather equipment and supplies.
2. Identify patient and inquire about allergies.
3. Provide privacy and safety for the patient.
 - a. Place patient in a position of comfort as long as the procedure permits.
 - b. Explain procedure to the patient.
4. Perform a history and physical exam of the eyes.
 - a. Patient history.

NOTE: An accurate history is needed in establishing a possible cause for the eye problem.

- (1) Mechanism of injury (if there is a history of injury)?
 - (a) Blunt trauma versus penetrating injury.
 - (b) Was there a projectile or missile injury? (Glass from a motor vehicle accident.)
 - (c) Thermal, chemical or laser burn?
- (2) Does the patient wear glasses or contact lenses?
- (3) History of eye disease or previous eye trauma/surgery?
- (4) Is there eye pain or loss of vision? If there is vision loss, is it in one eye or both?

NOTE: Ocular injuries are classified as penetrating or non-penetrating. Eye injuries are common in spite of the protection by the bony orbit.

- b. Physical exam.

WARNING: In cases of ocular burns, the eyes must be flushed first. Once the mechanism of the burn has been eliminated, then a visual acuity screening is accomplished.

- (1) Inspect and palpate in adequate lighting.

NOTE: Avoid putting pressure on the globe while performing the exam.

- (3) Visual acuity is the most important step in evaluating ocular problems.

Have the patient:

- (a) Read any available printed material.
- (b) Count fingers.

NOTE: If the patient is unable to count fingers, assess their ability to detect hand motion 1 to 2 inches in front of the eye.

- (c) Distinguish between light and dark.

CAUTION: When equipment, light and /or space do not permit a gross vision examination is recommended as opposed to no vision exam.

(d) Read the Snellen chart (eye chart). (See task 081-833-0193.)

NOTE: If the patient has corrective lenses, test without glasses first, and then test with glasses on.

(3) Note any drainage or bleeding from the eye.

(4) Inspect eyelids for:

(a) Ability to open wide and close completely.

(b) Edema.

(c) Discoloration.

(d) Foreign bodies.

(e) Inspect position of the lids in relationship to the eyeballs.

(5) Inspect conjunctiva and sclera for:

NOTE: Ask patient to look upward as you pull the lower lid downward. Then tell the patient to look down as you pull the upper lid upward.

(a) Note color of conjunctiva, (pink to dark color is normal).

(b) Inspect for erythema or exudate.

(c) Note color of sclera (white is normal).

NOTE: Patient's who have darker pigmented skin may have scattered areas of brown pigments, a normal finding, but this observation should be noted.

(6) Inspect cornea (should be clear and avascular).

(7) Inspect pupil-note any irregularity in the shape of the pupils.

NOTE: Unequal size of pupils (anisocoria) may be congenital (approximately 20% of normal people have minor or noticeable differences in pupil size, but pupillary reaction is normal.)

(a) Pupils should be equal, round, regular and reactive to light (PERRL).

(b) Assess extra ocular movement.

(8) Test pupillary reaction to light both directly and consensually.

(a) Dim the lights in the room so that the pupils dilate.

NOTE: Do not shine into both eyes simultaneously.

(b) Shine a penlight directly into one eye and observe the pupil constrict.

(c) Note consensual reaction response of the opposite pupil constricting simultaneously with the tested pupil.

5. Perform a history and physical exam of the ears.

a. Patient history.

(1) Ask patient about symptoms associated with ear problems, pain, swelling, redness, drainage.

(2) Ask patient about history of recent illness, upper respiratory infection (URI), sore throat, or recent swimming.

(3) Ask patient about any hearing loss.

- (4) Ask patient if they have a fever of 101° F or greater.
- (5) Ask the patient about possible embedded foreign objects in ear.

b. Physical exam.

NOTE: Physical examination consists of inspection, palpation and otoscopic examination of the ear.

- (1) Examine external pinna for erythema, swelling, deformity, trauma, or drainage from canal.
- (2) Auricle—Inspect the ear size, shape, symmetry, landmarks, color, and position on the head; palpate the auricle for tenderness and swelling.
- (3) Palpate the ear and mastoid process.
- (4) External auditory canal—inspect the external auditory canal (EAC) for discharge or earwax (cerumen).

CAUTION: Consult with your medical officer before completing an otoscopic exam on a child.

- (5) Using otoscope inspect the EAC and middle ear.

NOTE: Use the largest speculum the ear canal will accommodate.

- (a) Examine the normal ear first.
- (b) Hold the otoscope handle between your thumb and fingers; brace your hand against the patient's face.
- (c) Pull the auricle back and out on an adult patient.
- (d) Visualize the canal as you insert the speculum.
- (e) Slowly insert the speculum noting discharge, lesions, narrowing of the EAC, foreign bodies, or presence of cerumen (earwax).

NOTE: A healthy TM or ear drum may be pearly gray to amber in color. Redness indicates infection.

6. Perform a patient history and physical exam of the nose and sinuses.

a. Patient history.

- (1) Ask about recent upper respiratory infection symptoms, drainage, bleeding, congestion, trauma, pressure, recent dental problems, and pain.
- (2) Ask about any medications, prior history or chronic illnesses.
- (3) Ask about duration of symptoms.
- (4) Ask about any allergies or family history of allergies.

b. Physical exam.

- (1) Inspect for shape, size, symmetry, color, and presence of deformities or lesions.
- (2) Palpate the external nose for tenderness, swelling or masses.
- (3) Paranasal sinuses.
 - (a) Inspect the frontal and maxillary sinus area for swelling.
 - (b) Palpate the frontal and maxillary sinuses for tenderness.

NOTE: Only the maxillary and frontal sinuses are accessible to physical examination.

7. Perform a patient history and physical examination of the throat.

a. Patient history.

(1) Ask patient about recent symptoms, bad breath, sore throat, hoarseness, difficulty swallowing, and inability to open mouth.

(2) Ask patient about duration of symptoms, smoking habits and drooling.

b. Physical exam.

(1) Inspect and palpate the patient's lips for symmetry, color, edema and surface abnormalities.

(2) Inspect buccal mucosa, teeth, and gums.

NOTE: If patient wears dentures, have them remove their dentures and then open their mouth. Gums should have a slightly stippled, pink appearance with a clearly defined, tight margin at each tooth.

(a) Have the patient to open their mouth, stick out their tongue and say "ah."

(b) Use a disposable tongue blade and bright light to inspect mucous membrane of the entire mouth, teeth, posterior pharynx and tonsils.

NOTE: Mucous membranes should look pinkish-red, smooth and moist. Take note of any missing teeth, cavities, ulcerations and lesions.

(3) Inspect the tongue.

NOTE: The tongue should appear dull red, moist and glistening; should be smooth with increasing roughness.

(4) Inspect the oropharynx.

NOTE: The whitish hard palate should be dome-shaped. The pinker soft palate should be continuous with the hard palate.

8. Document findings on the SF 600.

Evaluation Preparation:

Setup: For training and evaluation, use a another Soldier as the simulated patient.

Brief Soldier: Tell the Soldier the simulated patient requires a history and physical examination of the HEENT.

Performance Measures	GO	NO GO
1 Gathered equipment and supplies.	_____	_____
2 Identified patient and inquired about allergies.	_____	_____
3 Provided privacy and safety for the patient.	_____	_____
4 Performed a patient history and physical exam of the eyes.	_____	_____
5 Performed a patient history and physical exam of the ears.	_____	_____
6 Performed patient history and physical exam of the nose and sinuses.	_____	_____
7 Performed a patient history and a physical exam of the mouth and oropharynx.	_____	_____
8 Documented findings on the SF 600.	_____	_____

Evaluation Guidance: Score each Soldier according to the performance measures. Unless otherwise stated in the task summary, the Soldier must pass all performance measures to be scored GO. If the Soldier fails any steps, show what was done wrong and how to do it correctly.

References**Required**

SF 600

Related

None